



ENGINEERING INTUITION: THE ATANDA COMMAND CENTRE

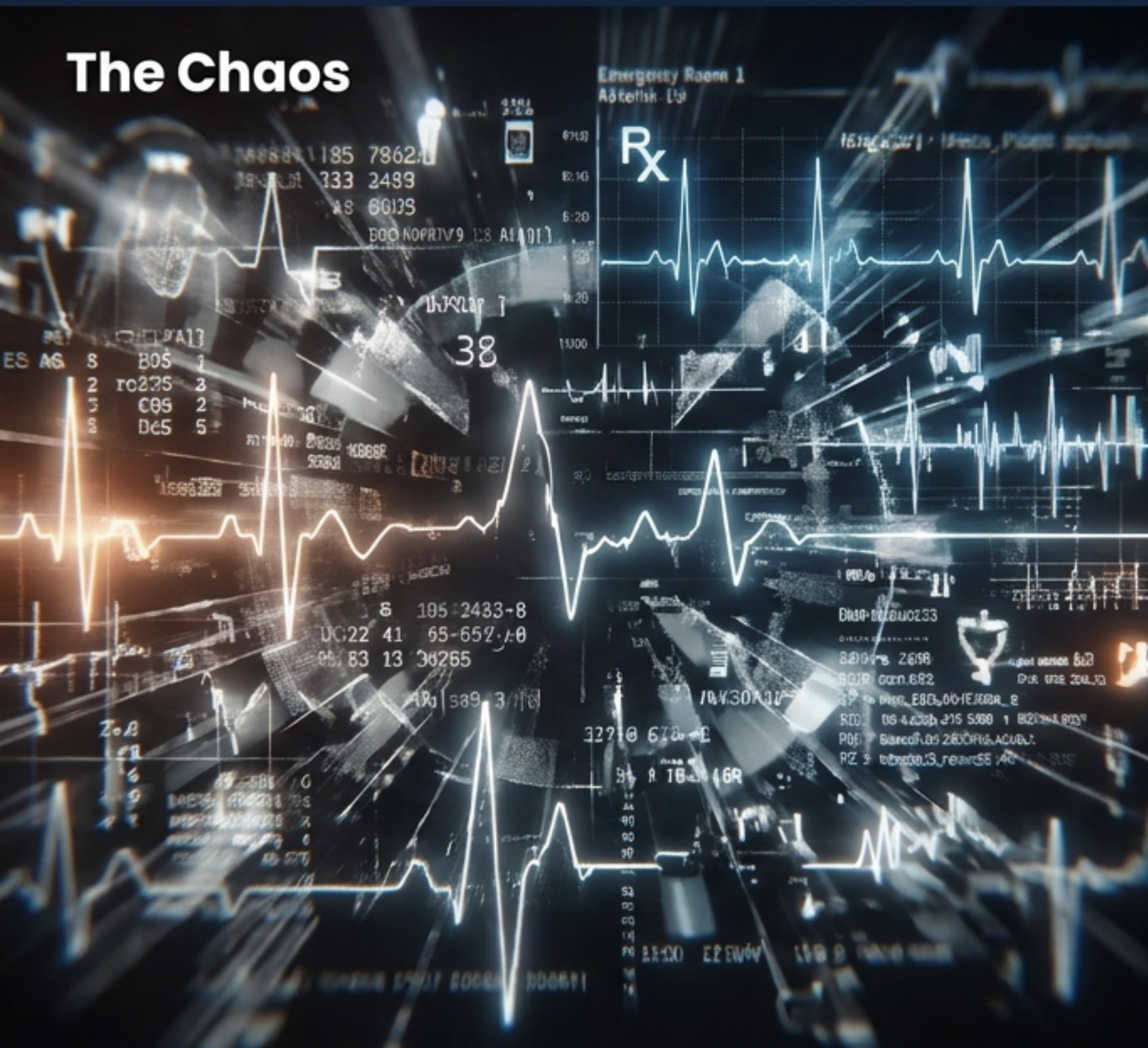
Introducing TRIAGE-DX and the Cognitive Co-Pilot Architecture for Starfleet Hospital.

Prepared by the Intelligent Systems Architect, ATANDA.

Emergency Rooms operate at the absolute edge of human cognitive capacity.



The Chaos



The Reality

12-18%

Early patient deterioration signals missed during manual triage in high-volume surge events.

18 Minutes

The current baseline average triage-to-treatment time in a busy ER.

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£15k – £22k

Annual administrative cost per institution in nursing time for retrospective triage documentation.

This is a **cognitive architecture** problem—and it is solvable with well-governed clinical AI.

An institution-grade instruction layer that safely orchestrates clinical data.



We do not build generic AI chatbots. The ATANDA Command Centre AI Harness is a highly governed 13-engine pipeline.

For Starfleet Hospital, this harness powers **TRIAGE-DX**: an AI triage decision-support agent that absorbs and sorts real-time telemetry patient data to produce a continuously updated priority ranking.

TRIAGE-DX
Application

ATANDA
Command
Centre
AI Harness
(13 Engines)

Starfleet
Hospital
Infrastructure
(HL7 FHIR /
Telemetry)



Governed Recommendation Only: Authority remains unconditionally yours.



Zero Autonomous Action

TRIAGE-DX operates under the GRO-GREY Ethics Class. It never prescribes treatments or acts independently.



Verified Trust

Every interaction requires real-time validation via the Credential Verification Service (CVS) to confirm licensed clinician identity.

The Anchor Rule

Every output is a recommendation. The final clinical authority rests entirely with you.

We extract, structure, and scale your clinical intuition.



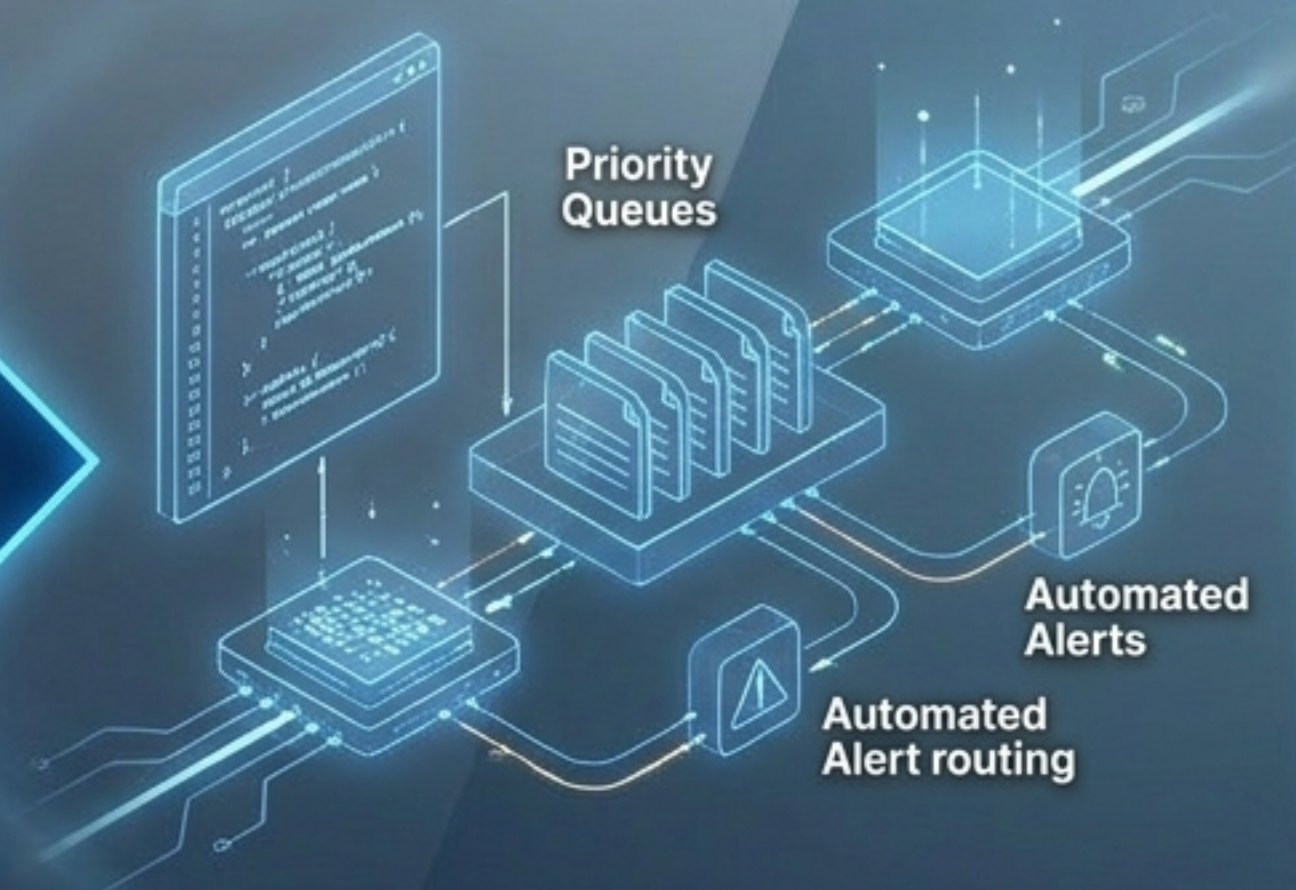
Human Know-How



Your hard-won professional instincts—how you instantly recognize a deteriorating patient across a crowded ER.

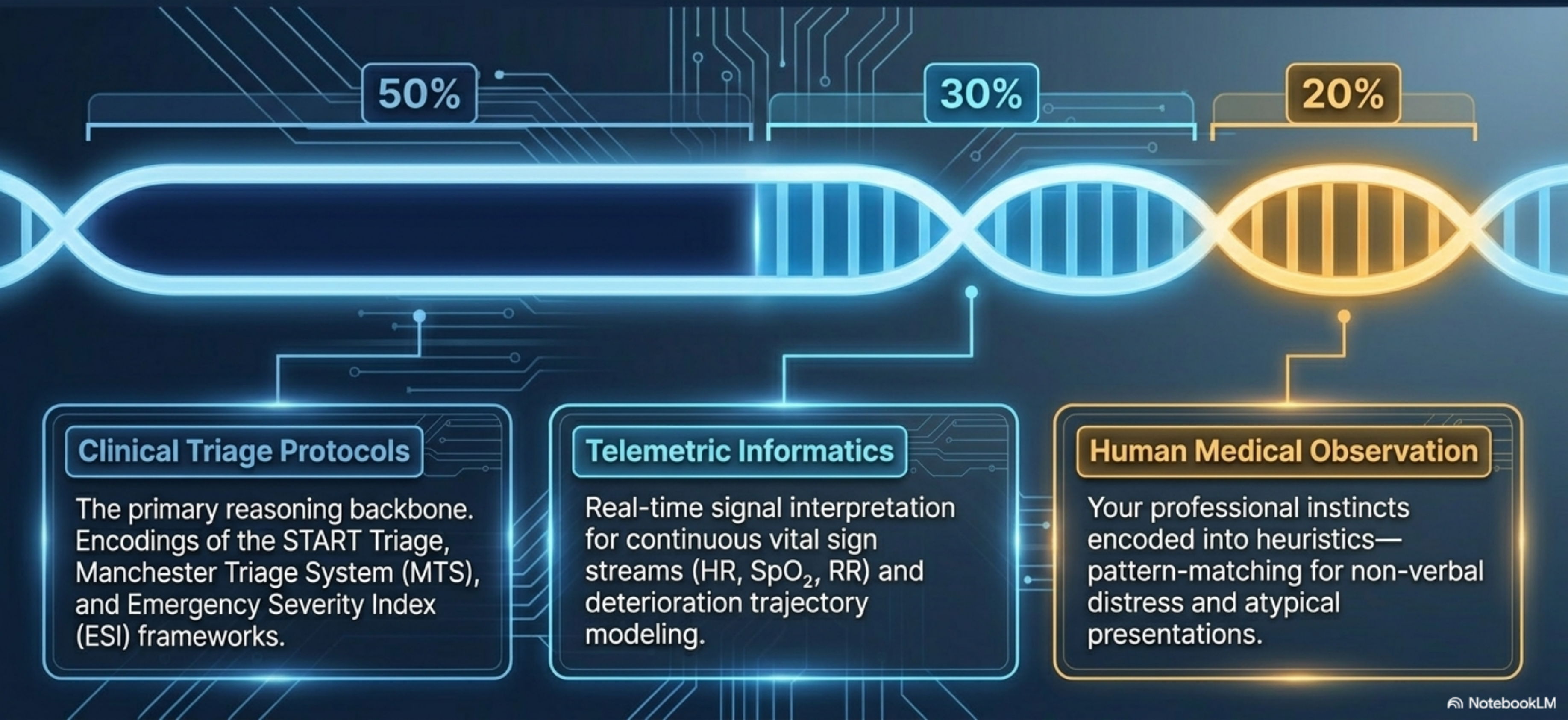
Cognitive Engineering

Cognitive Assets



Translated into deterministic, certified cognitive assets that work tirelessly at the speed of telemetry.

The Super Prompt Card (SPC) is the certified DNA of the AI.



Clinical Triage Protocols

The primary reasoning backbone. Encodings of the START Triage, Manchester Triage System (MTS), and Emergency Severity Index (ESI) frameworks.

Telemetric Informatics

Real-time signal interpretation for continuous vital sign streams (HR, SpO₂, RR) and deterioration trajectory modeling.

Human Medical Observation

Your professional instincts encoded into heuristics—pattern-matching for non-verbal distress and atypical presentations.

Seamless integration within the institution's existing network perimeter.



Starfleet Infrastructure



Epic/Cerner
EHR



Vital Sign
Sensors

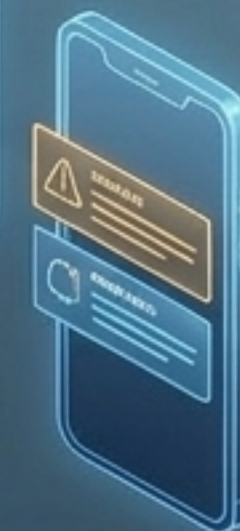
Standardized Ingestion: Native consumption of HL7 FHIR R4 continuous telemetry data.

ATANDA Harness



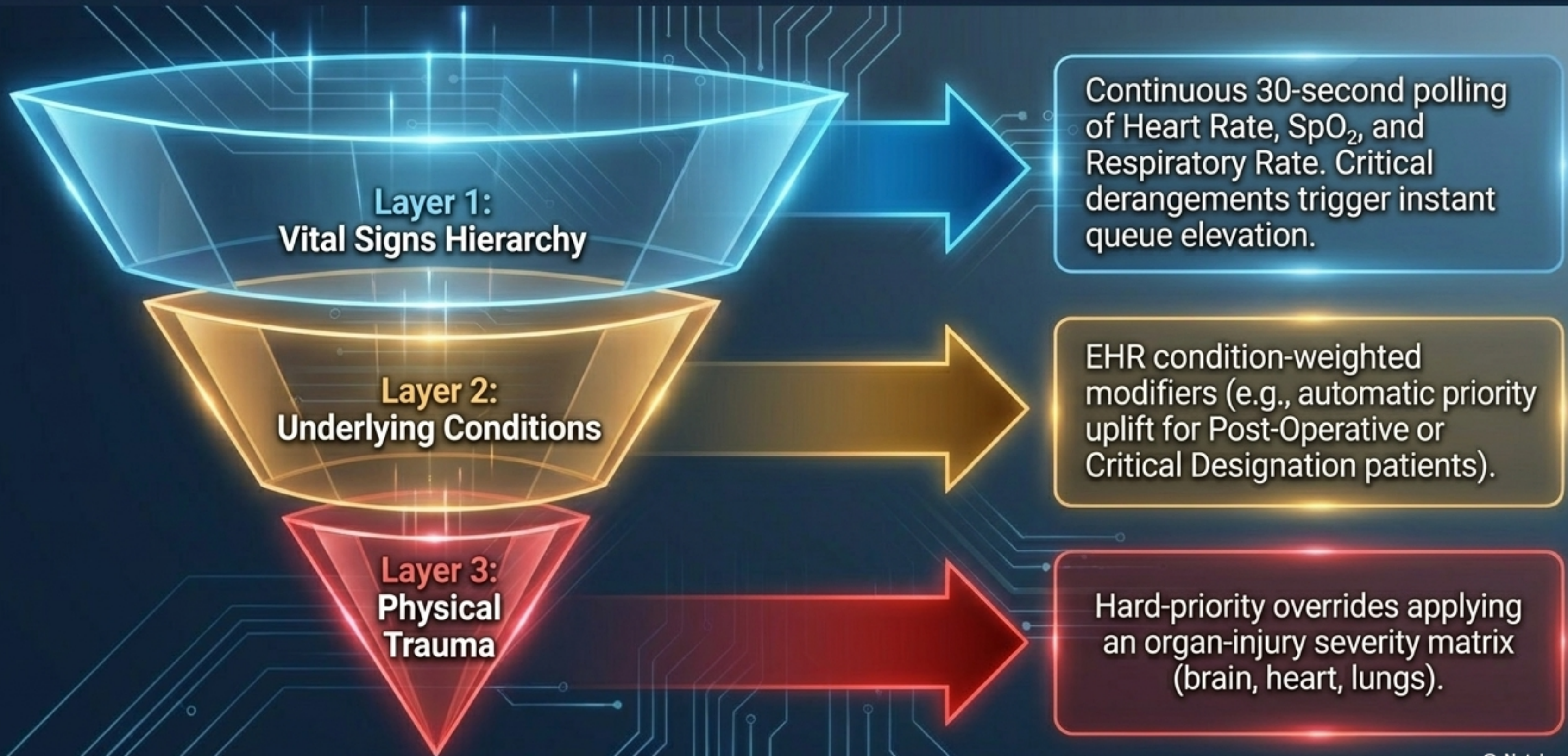
Total Data Isolation: Operates strictly within the hospital's secure network; patient data never transits public clouds. Uses Read-Only HL7 FHIR R4 API Proxy.

Clinical Output



Frictionless Output: Pushes ranked recommendations directly to the clinical dashboards and mobile alerts you already use.

A strict 3-Layer deterministic engine translates medical doctrine into code.



Sub-Agent Gates manage critical escalations and conflict resolution.



Occupying the vital gap between passive analytics and unregulated AI.



	Legacy Broad EHR Analytics	Single-Metric Alerts	ATANDA TRIAGE-DX
Multi-Patient Surge Prioritization	Limited	✗ No	✓ Yes
Real-Time Latency (<30s)	✗ No	✓ Yes	✓ Yes
Inherent Ethical Governance	N/A	N/A	GRO-GREY Protocol
Clinician Override Audit Trail	Manual	✗ No	Immutable / Automated

Institutional governance, measurable ROI, and regulatory readiness.



Regulatory Readiness



Architected specifically for EU AI Act High-Risk conformity and strict GDPR data processing compliance.

Risk Reduction



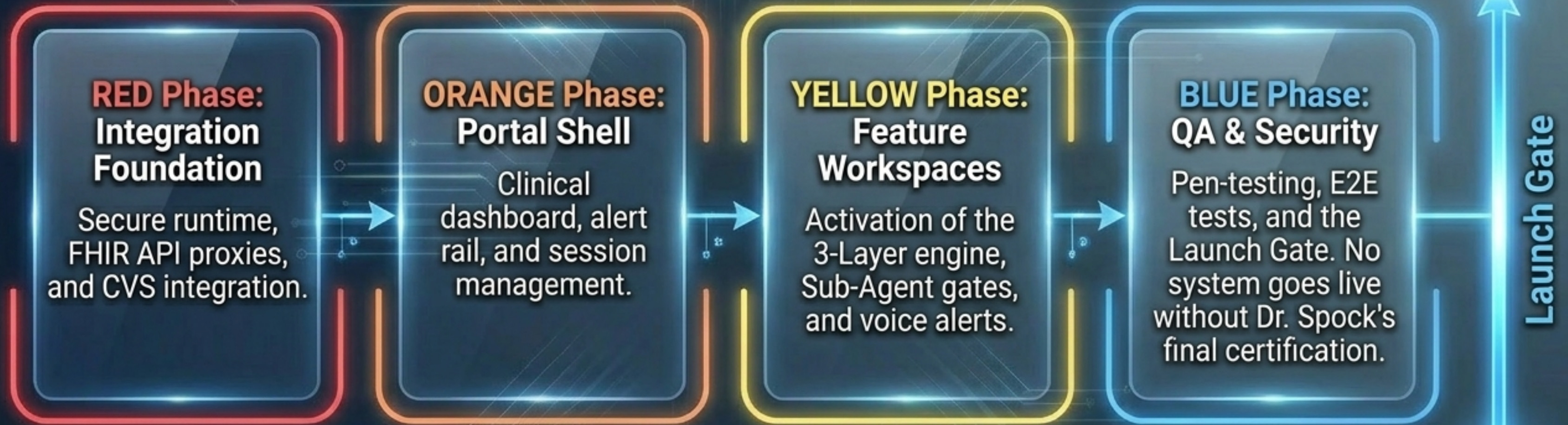
Targets a 35% reduction in triage-to-treatment time, moving from 18 minutes to < 12 minutes.

Measurable ROI



Automated immutable logging eliminates 4-6 hours per week of manual retrospective documentation per nurse.

A controlled, certified pathway to live deployment.



Reclaiming your cognitive space.



TRIAGE-DX

PRIORITY	PATIENT ID	CONDITION	WAIT TIME
P1	- 1245A	- CHEST PAIN	2m
P2	- 1328B	- FRACTURE	5m
P3	- 1330C	- CHEST PAIN	2m
P4	- 1345D	- FRACTURE	5m
P5	- 1336D	- RVLATHEE	2m

The goal of ATANDA and TRIAGE-DX is not to replace your judgment. It is to **eliminate the noise.**

By engineering your intuition into the system, our ultimate target is to reduce your NASA-TLX cognitive load scores by 40% during a multi-patient surge.

We manage the data queue, so you can return to what you do best: treating the patient in front of you.